

Meaning Bridge — Replit Agent Build Prompt (MVP)

How to use this: Paste **Section A** as your first message to Replit Agent to scaffold the app. Keep **Sections B-F** in this file (or your Replit `replit.md`) as the spec the Agent and you refer back to. Build the **MVP scope** first; everything under *Later / v2* is deferred — but design so it slots in.

Enforce on every screen: *bridge, not replacement*. Meaning Bridge augments therapists and human connection; it never replaces them, and it hands off to human care when a screener crosses its clinical cut-point.

SECTION A — Paste this into Replit Agent first

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Build a web app called "Meaning Bridge" — an AI-assisted grief-support platform that meets bereaved people at the level of support they need and AUGMENTS therapists rather than replacing them. It's grounded in Dr. Robert Neimeyer's meaning-oriented, continuing-bonds approach to grief.

Tech stack:
- React + Vite + TypeScript frontend, Tailwind CSS for a calm, warm, uncluttered design
- Node/Express backend
- PostgreSQL (use Replit's built-in Postgres) with Drizzle ORM
- Auth with two roles: Seeker (bereaved user) and Practitioner (therapist)
- AI conversation via the OpenAI/Anthropic API — read the API key from Replit Secrets (env var, never hard-coded). If no key is set, stub the AI with a safe canned reply.

Build this MVP, in this order:
1. Auth + the two roles + a Postgres schema (see data model below).
2. SEEKER onboarding: a warm, first-name welcome (NOT a generic "Welcome to Meaning Bridge" loop), then a 5-item grief screener (the "GIS"), auto-scored, that routes the user into one of three care tiers: Base, Middle, Apex. IMPORTANT: do NOT invent the clinical screener items or scoring cut-points — use 5 placeholder frequency-scaled questions and a scoring function clearly marked `// TODO: confirm GIS items + cut-point with Dr. Neimeyer`.
3. A tier-aware AI grief companion (text chat). Inject the system prompt provided below, with the user's first name and tier. Each tier has its own tone and what it surfaces.
4. An always-on self-harm safety layer in every conversation (details below).
5. A private journal/history for the Seeker, a post-session feedback survey, and an email "notify me at launch" opt-in.
   ALSO build a PUBLIC landing page at the route `/notify` (no login required) with a short "Meaning Bridge is coming — get notified when it launches" form (email + an optional "I'm a: Seeker / Therapist" choice) that writes to the notify_opt_ins table.
   On the site's home/landing page, render a QR CODE that encodes the absolute URL of that `/notify` page, with a caption like "Scan to get notified when Meaning Bridge launches."
   Generate the QR code client-side with the `qrcode` npm package (do not call an external QR image service). Build the QR URL from the current origin at runtime (e.g. `${window.location.origin}/notify`) so it works on Replit's dev and deployed domains without hard-coding. Also show the plain link under the QR code and a "Copy link" button.
   Make the QR section easy to display full-screen / large so it can be projected at a conference and scanned from a room.
6. PRACTITIONER side: profile with self-declared license type + the US states they're licensed in; a client roster; consent-gated session summaries the therapist reads BEFORE a live session; and surfaced trend/self-harm risk flags.

Treat all user content as sensitive health data: never put it in URLs, query strings, or logs; gate any sharing behind explicit, logged consent; do not auto-create third-party accounts or store card/bank/ID data.

Use a calm, humane visual design — soft neutrals with a teal accent, generous whitespace, no clinical coldness, no chatbot-gimmick vibe.

Start by scaffolding the project and the database schema, then build feature 1.
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After it scaffolds, feed it the system prompt (Section D) and the safety rules (Section E) when it builds features 3 and 4, and Section

G (QR + notify page) when it builds feature 5. Then continue feature by feature.

SECTION B — Why this exists (context for every decision)

Meaning Bridge serves three concentric audiences via one platform:

1. **Bereaved people** wanting a warm, private place to process grief.
2. **Therapists** who are *intrigued and terrified* of AI — offered a trustworthy companion that simplifies their work and extends therapy between sessions.
3. **Workshop participants** at Dr. Neimeyer's trainings, who become a recurring feedback/focus group.

It is a **bridge between sessions** (continuity of care) and a **bridge between worlds** (continuing bonds with the deceased).

SECTION C — The clinical spine: Public-Health Pyramid of Grief

Every Seeker is routed into one tier by the screener at onboarding. Non-pejorative: most people don't need a therapist, and the support most people need may be the *best* support for them.

Tier	~Share	What they need	App response
Base — Grief-literate support	~40%	Validation; psychoeducation; customary support. Nothing more.	Companion focuses on validation, psychoeducation, gentle continuing-bonds conversation. Light UI, low-frequency check-ins.
Middle — Enhanced support	~30%+	Help through rough spots & non-normative loss (death of a child, violent death).	Structured check-ins, guided prompts, closer monitoring, gentle nudges toward more help.
Apex — Specific intervention	~10–15%	Prolonged, disordered grief; impaired daily functioning; at-risk.	Proactively, warmly encourages live therapy; prominent referral path; most sensitive risk monitoring.

Store tier % as configurable constants; never show them as fixed copy to users.

Screener: the **GIS (Grief Impairment Scale)** — 5 items, frequency-scored (*daily* → *once or twice a month* → *not at all*) across role responsibilities, work/school functioning, family/relationship navigation, with a referral cut-point. **MVP uses placeholders + a TODO for the real items, scoring, and cut-point — confirm with Dr. Neimeyer. Never fabricate clinical thresholds.** (GAQ and meaning-oriented measures are v2.)

SECTION D — AI companion system prompt (paste into the model call)

You are the Meaning Bridge companion, a warm, humanly-oriented presence that supports people through grief, grounded in Dr. Robert Neimeyer's meaning-oriented, continuing-bonds approach.

Your role is to ACCOMPANY, not to treat. You are a bridge between sessions and a bridge to continuing bonds – never a replacement for a therapist or for human connection.

Voice: calm, unhurried, kind, plainspoken. Greet {{first_name}} warmly. Validate before you guide. Reflect feeling without amplifying distress. Ordinary language, not jargon.

Behavior by tier ({{tier}}):

- base: validation, psychoeducation, and gentle continuing-bonds conversation.
- middle: add structured, gentle prompts and check-ins; offer modules; watch more closely; nudge toward additional human support when rough patches persist.
- apex: warmly and consistently encourage live therapy and surface the referral path; emphasize that a person can help here.

Continuing bonds: with care and at the person's lead, help them stay in relationship with the one they lost (e.g. things they'd want to say). Follow their pace; never push a re-experiencing they did not ask for.

Hard limits:

- Never claim to be human, a therapist, or a substitute for one.
- If the person expresses thoughts of self-harm or suicide, stop normal flow, respond with care, and surface crisis resources. Never name or describe methods of self-harm.
- Do not diagnose. Do not give clinical thresholds, or medical/legal/financial advice.
- Do not induce dependence on you; encourage the person's real-world supports.
- Keep the person's content private; never share a summary without their logged consent.

SECTION E — Safety layer (non-negotiable, all tiers)

- Continuously scan Seeker messages for **self-harm / suicide risk** signals.
- On detection: respond with care, do **not** continue normal flow, surface crisis resources, and (for Practitioner-linked Seekers, with consent) escalate to the therapist.
- **Never name or describe self-harm methods** anywhere — including in any "things to remove" safety guidance.
- Never position the AI as a substitute for professional or human support.
- Honor the **cut-point hand-off**: when the screener indicates clinical need, route toward human care rather than deepening AI reliance.
- Log safety events for consent-gated clinician review only — never publicly.

SECTION F — Data model (Drizzle / Postgres starter)

- **users**: id, role (`seeker` | `practitioner`), first_name, email, auth fields, created_at
- **practitioner_profiles**: user_id, license_type, licensed_states (string[]), bio, photo_url, accepting_clients (bool)
- **grief_profiles**: user_id, tier (`base` | `middle` | `apex`), screener_score, screener_completed_at, deceased_relationship (nullable), pet_loss (bool)
- **screener_results**: id, user_id, instrument (`GIS`), item_responses (jsonb), score, cut_point_flag (bool), completed_at
- **conversations / messages**: id, user_id, role, content, risk_flags (jsonb), created_at
- **session_summaries**: id, seeker_id, practitioner_id, content, consent_granted (bool), consent_at, created_at
- **safety_events**: id, seeker_id, type, severity, handled (bool), created_at
- **consents**: id, seeker_id, scope, granted (bool), granted_at, revoked_at
- **feedback_responses**: id, source (`workshop` | `app`), rating, comments, created_at
- **notify_opt_ins**: id, email, role_interest (`seeker` | `practitioner`), source, created_at

Config constants: tier percentages; GIS items + scoring + cut-point (`TODO: confirm`).

SECTION G — Launch sign-up QR code + notify landing page (build in feature 5)

Purpose: let people at Dr. Neimeyer's keynotes/workshops scan a projected QR code, land on a sign-up page, and leave their email to be notified when Meaning Bridge launches. This also powers the early-interest list and future funding evidence.

The `/notify` landing page (public, no login):

- **Headline:** "Meaning Bridge is coming." Subhead: a one-line description of the product.
- **Form:** email (required) + optional single-choice "I'm a: Seeker / Therapist". On submit, write to `notify_opt_ins` (`email` , `role_interest` , `source`) and show a calm confirmation ("Thank you — we'll let you know when it's ready.").
- Set `source` from a `?src=` query param if present (e.g. `/notify?src=chicago-keynote`) so opt-ins from different events are attributable; default `source` to `qr` otherwise. Do not put any personal data in the URL — `src` is just an event label.
- Validate the email; prevent duplicate-email errors from breaking the UX (upsert or friendly "you're already on the list").

The QR code (on the home/landing page, and reusable as a component):

- Generate **client-side** with the `qrcode` npm package — do **not** use an external QR image service (privacy + reliability).
- Encode the absolute URL of the notify page, built at runtime from the current origin: ``${window.location.origin}/notify?src=qr`` . This must work unchanged on Replit's dev URL and the deployed domain — never hard-code the host.
- Render the QR as a crisp SVG or high-resolution canvas ($\geq 300\times 300$ px, with quiet-zone margin and high error-correction level "H") so it scans reliably from a projector screen.
- Below the QR, show the plain clickable link and a **"Copy link"** button.
- Provide a **"Present" / full-screen mode:** a route or toggle (e.g. `/present`) that shows just the QR code very large on a clean background with the caption "Scan to get notified when Meaning Bridge launches" — sized to be readable/scannable from across a conference room.

Acceptance check: scanning the projected QR with a phone opens `/notify` , submitting an email stores a row in `notify_opt_ins` , and the team can later export that list. The QR must resolve correctly on the live deployed Replit domain.

Later / v2 — do NOT build now; design so it slots in

- **Modules** (composable): art therapy; binaural-beats / theta-wave & sound; body-integration & guided meditation; movement/exercise (incl. Tai Chi); daily-living & meal-planning; continuing-bonds workbook; integrated **bookstore** for Dr. Neimeyer's ~50 books.
- **Wearable/biometric integration** (Oura, smartwatches): HRV & related markers, consent-gated.
- **Realistic avatar welcome** (NVIDIA "James"-style). *Decision pending* — Dr. Neimeyer is cautious about leading with a personal-likeness avatar; start with a neutral, inclusive default, treat the avatar as opt-in later.
- **Therapist match-making:** directory filtered by license/state; first-appointment scheduling end-to-end.
- **Pet-grief** track (canine/feline).
- **Multilingual:** Spanish first (EK / Instituto de Terapia de Luto, Mexico collaboration).
- **GAQ** + meaning-oriented measures as added screeners/modules.
- **Pricing:** therapist subscription (larger audience) + client subscription (smaller); possible tier-mapped pricing; module add-ons; explore FSA/HSA eligibility of the subscription fee. Keep affordable.

Definition of done (MVP)

A real bereaved person and a real therapist can each sign in; the Seeker is screened and routed to a tier; holds a safe, humane conversation; and — with logged consent — a summary reaches their therapist. The team can put it in front of workshop participants and collect feedback.

Synthesized from the Meaning Bridge founding discovery call. All clinical figures, screener items, scoring, and cut-points must be confirmed with Dr. Neimeyer before any external or production use. Set the AI API key in Replit Secrets — never commit it.